Methoxyflurane (Penthrox®) inhaler
Information for patients, parents and carers

What you need to know before your child is given Penthrox.

The majority of patients can use Penthrox, and a doctor or nurse will ensure that it is safe for your child before prescribing it.

Penthrox should not be used if:

- Your child is allergic or has had a bad reaction to any anaesthetic given through a mask.
- Your child or family has a history of malignant hyperthermia. This is a condition where a very high fever occurs after being given an anaesthetic.
- Your child has serious kidney or liver problems.
- Your child has significant heart or circulatory problems.
- Your child has recently taken or is taking certain medications such as strong painkillers and some types of antibiotics, anticonvulsants, antivirals and sedative antihistamines. Please inform your doctor about any regular or recent medication, and they will advise accordingly.

Helping your child to use Penthrox.

Before use, a nurse or doctor will show your child how to use the device correctly.
Penthrox is self-administered, which means your child will have control of when to use it.
It is simple to use with the mouthpiece held in between the teeth and the lips closed around it.
Your child will be asked to breathe normally in and out through their mouth.
It is important that your child breathes out through the mouthpiece as this collects any exhaled drug vapour rather than it being released into the air.
It usually takes 6-10 breaths before the medicine starts working.
Penthrox has a strong fruity taste and smell. The first few breaths often taste very strong and normally cause patients to cough, but the coughing sensation settles after the first few breaths.

Once Penthrox has started working, your child can breathe it in from time to time when they feel they need more pain relief. When used like this, an inhaler will typically last around an hour.

If we need to carry out a painful procedure (such as applying a cast, dressing a burn, treating a fracture or dislocation or closing a wound), we may ask your child to use the inhaler continuously for a short period of time to provide strong pain relief whilst we perform the procedure. When used like this, an inhaler will typically last around 20 minutes. Pain relief will continue for several minutes after stopping Penthrox.

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What are the possible side effects?

The most likely side effects can include:

- Headache
- Dizziness
- Dry mouth
- Drowsiness
- Sickness
- Tingling skin
- Double vision
- Flushing of the skin or skin irritation

These side effects normally wear off within a few minutes of stopping use. If your child becomes drowsy, they will be unable to hold the mouthpiece to their mouth, stopping them from inhaling the vapour and allowing the effect to wear off. If your child becomes drowsy or unresponsive, please alert a member of staff.

Serious side effects are rare but include allergic reactions, liver damage or kidney problems. If your child becomes unwell after using Penthrox please seek medical attention and inform staff that your child has recently used Penthrox.

This leaflet only gives general information. You must always discuss the individual treatment of your child with the appropriate member of staff. Do not rely on this leaflet alone for information about your child's treatment. This information can be made available in other languages and formats if requested.

If you have any concerns that your child may be experiencing problems relating to the medication that they have received then please contact the Emergency Department on 0141 452 4055 to discuss the issues with a senior doctor or nurse.